



Perinatal Psychiatry Consultation Service Referrals (Acute) Guideline

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

Background

- The RKPG Mental Health Perinatal Psychiatric Consultation Service is community based team embedded in the Peel and Rockingham Mental Health Clinics. It is not a separate team, but provides specialist input to the community mental health teams. It consists of a sessional academic perinatal Psychiatrist and a trainee in psychiatry, assisted by community mental health clinicians. The service has the following aims:
 - * Provide psychiatric management of pregnant women with mental illness in consultation and liaison with obstetric services (including but not exclusive to Rockingham General Hospital (RGH), Peel Health Campus (PHC), Fiona Stanley Hospital (FSH) and King Edward Memorial Hospital (KEMH), General Practitioners (GP's) and General Practitioner Obstetricians (GPOs) and relevant primary care services.
 - * Provide ongoing psychiatric care for women with mental illness in the postpartum period and their families (up to 12 months post-delivery) in conjunction with community mental health services as well as relevant non-government organisations (NGO's).
 - * Provide preconception counselling for women with enduring mental illness who are active patients with the community teams.
 - * Provide specialist advice to GPs, Mental Health and Obstetric services on the use of psychopharmacological interventions in pregnancy and breastfeeding.
- There is a single point of entry for all new referrals from external agencies, which is through the 'Assessment Treatment Team Triage', using the standard referral form. Once assessed and deemed appropriate, the Assessment Treatment Team (ATT) Triage will liaise with the perinatal team and patients are offered a psychiatric review. Following the review, patients are discussed at the ATT clinical review and admitted

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to either ATT or Clinical Treatment Team (CTT), depending on the referrals and the clinical need, to be co-managed with the Perinatal Psychiatric Consultation Service, or discharge if appropriate.

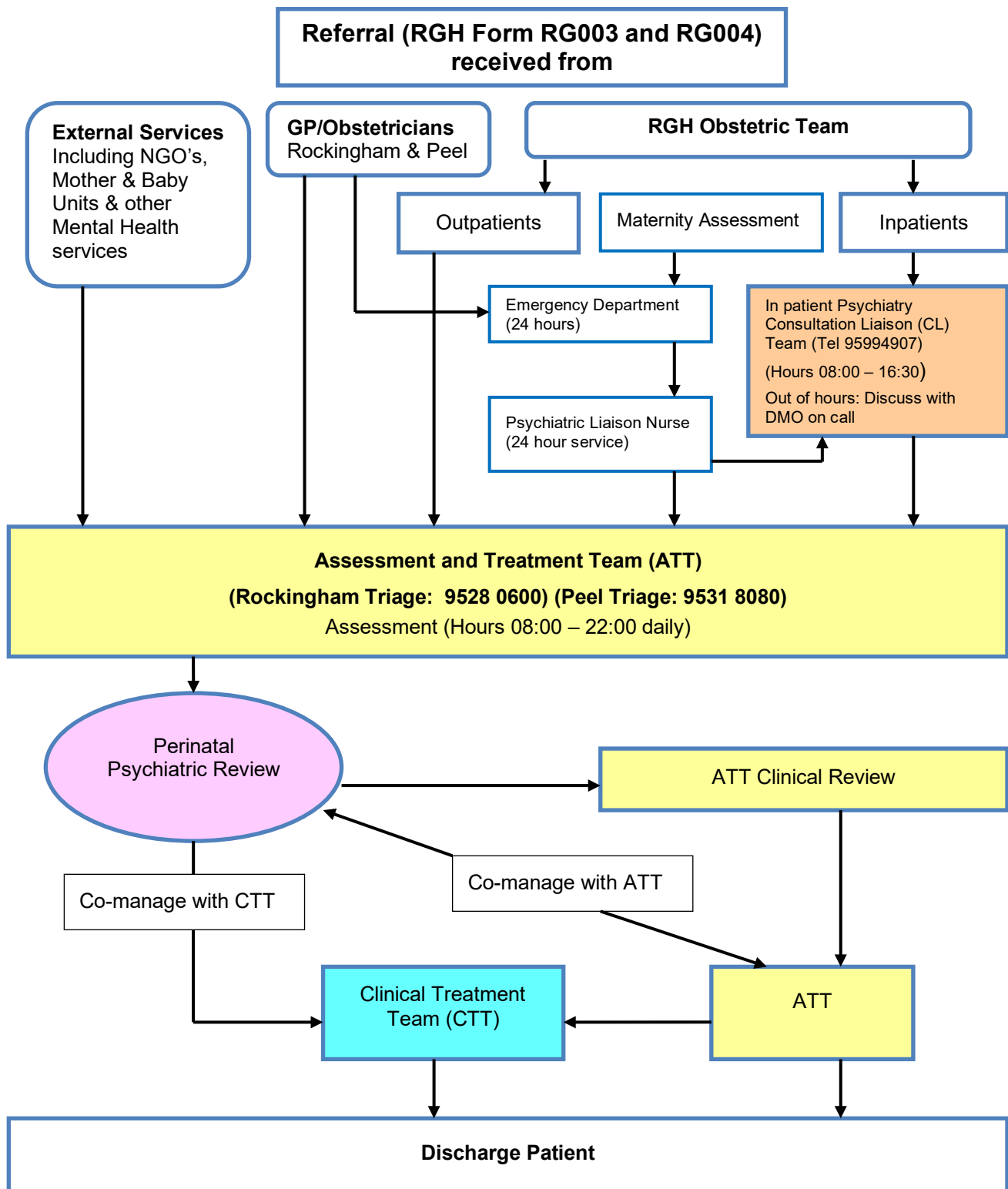
- The Perinatal Psychiatric Consultation Service also provides specialist support for existing patients of CTT and these referrals can be made directly to the Perinatal Psychiatrist.

Collaboration with RGH Obstetrics

- In partnership with RGH Obstetric Unit the Perinatal Psychiatric Consultation Service provides a number of strategies to better identify at risk pregnant women, and enhance the referral process:
 - * Accept direct referrals or queries from the RGH Obstetric Team, and Social Work, by email, and liaise with the ATT triage and assessment to organise assessments as necessary as per the single point entry highlighted in the flowchart.
 - * Provides liaison with the maternity Social Worker in identifying and supporting at risk women Take part in the regular Obstetrics Complex Care meetings.
- Following assessment in the maternity unit, the midwife will document that a referral has been sent to the Perinatal Psychiatric Consultation Service in the National Women Held Pregnancy Record (NWHPR). Documentation should include a plan of care if the patient has been reviewed by the Clinical Liaison Team or Psychiatric Liaison Nurse.
- The Obstetric team should consider making an appointment for the women with her community GPO following the referral to the Perinatal Psychiatric Consultation Service, to maintain optimal communication.
- For **inpatients** of RGH in working hours (08:00 - 16:30hrs) referrals are made to the RGH Psychiatric Consultation Service and pending appropriate assessment and intervention by the team follow up can be arranged in the community as per the pathway.
- For **inpatients** of RGH, out of hours assessments are discussed with the Duty Medical Officer (DMO) who, if needed, will discuss with the Mental Health Consultant Psychiatrist on call.
- For patients attending for antenatal care assessment (RGH **outpatients**) that are identified as requiring mental health input, the process depends on the acuity of the presentation. If immediate psychiatric assessment is required, the patient will need to be referred to RGH ED, with appropriate liaison with the Psychiatric Liaison Nurse (PLN). Otherwise, a referral can be made to ATT (Rockingham 95280600, Peel 95318080), who can make contact with the patient within 24-48 hours. Non-urgent referrals are made using the pathway discussed.

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Flowchart for Perinatal Psychiatry Consultation Service Referrals



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References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>

National Standards



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